



Prescribing Policy and Procedure

Scope

Site	Service/Department/Unit	Disciplines
SCGH and OPH (i.e. SCGOPHCG)	Organisation Wide	All Staff

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Introduction

Prescriptions that are illegible, ambiguous, or incomplete are a major contributing factor in many medication errors. This policy and procedure defines standards for safe prescribing by all prescribers at Sir Charles Gairdner Hospital (SCGH) and Osborne Park Hospital (OPH). Standards are in accordance with medication safety principles and the [Statewide Medicines Formulary](#).

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☐	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☒
Other:	☐		

Definitions

ARTG	Australian Register of Therapeutic Goods
Brand name	Name the pharmaceutical company assigns to the medication
Electronic Prescribing and Medication Administration (EPMA) Software	An electronic prescribing system alternative to paper-based medication charts, e.g. MOSAIQ.
Formulary	A list of approved medications available for prescribers. The State Medicines Formulary was adopted as the SCGOPHCG Formulary (refer to section 14 for details).
Generic name	The active ingredient name (the chemical that makes the medicine work)
Individual Patient Approval (IPA)	An electronic application utilised to gain access to or prescribe: • Non-formulary medicines • Formulary medicines for non-approved indications • Formulary medicines for indications outside of SCGOPHCG restrictions.
MMPC	Monitored Medicines Prescribing Code
MTC	Medicines and Therapeutics Committee
PBS	Pharmaceutical Benefits Scheme
Prescribe	To issue a prescription to instruct a pharmacist to dispense a medicine, or to issue an order (written or verbal) for a health professional to administer a medication.
SAS	Special Access Scheme
Specialised Medication Charts	Medication charts designed to improve safe prescribing of high-risk medications (e.g. Subcutaneous Insulin Order and Blood Glucose Monitoring Form, WA Anticoagulation Medication Chart)
TGA	Therapeutic Goods Administration
WA HMC	WA Hospital Medication Chart
WA Individual Patient Approval System (WAIPAS)	A state-wide Individual Patient Approval (IPA) system, known as WAIPAS, used to apply for, process and approve IPAs
webPAS	Patient Administration System

Policy Principles

Only health professionals that hold prescribing rights within their scope of practice and endorsed at SCGOPHCG are permitted to prescribe medicines:

- Medical Practitioners
- Nurse Practitioners
- Pharmacists, in accordance with Pharmacist Initiated Medicines and/or Partnered Charting Pharmacist Programs (also known as Partnered Pharmacist Medication Charting PPMC)
- Endorsed Podiatrists
- Dieticians

All prescriptions written within SCGOPHCG are to meet the minimum requirements set under:

- Medicines and Poisons Act 2014
- Medicines and Poisons Regulations 2016
- [Monitored Medicines Prescribing Code 2024](#)
- [WA Medication Chart Policy](#)
- WA Health Mandatory Policy MP 0077/18 [Statewide Medicines Formulary Policy](#)
- WA Health Mandatory Policy MP 0084/18 [Credentialing and Defining Scope of Clinical Practice Policy](#)

Prescriptions or medication orders must be documented on SCGOPHCG authorised stationary or electronic prescribing and medication administration software. See [Management of Prescription Stationery Policy](#).

All prescriptions must be issued in direct association with a registered episode of care for a patient receiving treatment within SCGOPHCG.

A prescription on approved hospital stationery is required for administration of:

- Scheduled medicines
- Non-scheduled medicines
- Intravenous and subcutaneous fluids
- Medical gasses
- Blood products
- Parenteral Nutrition

All prescribing must comply with requirements set out in the State Medicines Formulary (SMF). For non-formulary medications prescribed via Individual Patient Approval (IPA), all prescribers must be in line with the [Medicines and Therapeutics Formulary Policy](#)

When non-PBS subsidised medicines are prescribed in accordance with the SMF, a maximum of ONE month's supply with FIVE repeats (total of SIX months therapy) may be prescribed. Repeat supplies for these medications can be dispensed and supplied by the outpatient pharmacy.

1. Prescribing (General Requirements)

1.1 Prescribing Forms & Stationery

Valid prescription forms must be authorised by the Medicines and Therapeutics Committee (MTC) prior to use within the hospital. Forms must also be endorsed by the Forms Committee, having been issued a valid Medical Record Number.

All medication charts are to be ordered by clinical areas using only approved suppliers. No charts are to be photocopied for clinical use or printed from source files such as PDF, unless approved by the MTC.

- WA Medication Chart Policy mandates the implementation of a suite of standardised medication charts for prescribing and administering medications for inpatient care. The use of standardised medication charts leads to consistent and efficient documentation, and the safe prescription and administration of medication. This includes:
 - [WA-Hospital-Medication-Chart-Short Stay-Adult-Chart](#)
 - [WA-Hospital-Medication-Chart-Long Stay-Adult-Chart](#)
 - [WA-Paediatric-Hospital-Medication-Chart-Short-Stay](#)
 - [WA-Paediatric-Hospital-Medication-Chart-Long-Stay](#)
 - [WA-Anticoagulation-Medication-Chart-Adult Chart](#)
 - [WA-Clozapine-Initiation-and-Titration-Chart-Adult-Chart](#)
 - [WA-Agitation-and-Arousal-PRN-Chart-Adult-Chart](#)
 - [WA-Intramuscular-Long-Acting-Injection-Chart-Depot-Antipsychotic-Adult-Chart](#)
- Certain high-risk medicines are approved on specialised medication charts that must be approved by MTC for use at SCOPHCG such as the insulin subcutaneous order and blood glucose record chart and intravenous fluid chart.
- When a specialised medication chart is in use, this must be cross-referenced by completing a tick box on the front of the WA HMC at the time of initiation of the specialised medication chart.

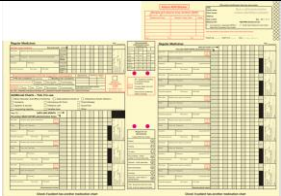
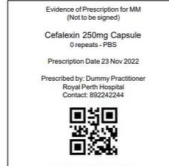
Figure 7: Cross-referencing tick box on front of WA HMC for specialised charts

Additional charts	☐ Variable dose		
☐ IV fluid	☐ BGL/insulin	☐ Acute pain	☐ Other
☐ Palliative care	☐ Chemotherapy	☐ Anticoagulation	

Other prescription forms must be ordered and supplied by the Pharmacy department, having the hospital approval numbers and specific site details. See [SCGOPHCG Management of Prescription Stationery Policy](#) for further details. Prescriptions can only be dispensed at the site at which they were issued unless a Shared Care Dispensing agreement is in place.



1.2 Prescription types & settings

Platform	Format	Outpatient (Hospital Pharmacy)	Outpatient (Community Pharmacy)	Inpatient	Discharge	Leave
Medication Charts		No	No	Yes	No	Yes
Paper Handwritten		In the event of a code yellow, as part of downtime procedures, only	In the event of a code yellow, as part of downtime procedures, only	No	In the event of a code yellow, as part of downtime procedures, only	No
NaCS Printed		No	No	No	Yes	No
eMedication Printed		Yes	Yes	No	No	No
eMedication Electronic (SMS/email Token)		No	Yes	No	No	No
Clinical Trial Prescription	Variable paper form	Yes (In DD Block)	No	Yes	Yes	No



2. Prescription details

All prescriptions must specify the following details:

- medicine (generic name)
- dosage form and strength
- dose, instructions for administration including frequency and route
- diluent where applicable
- indication
- rate and/or concentration where applicable
- the date treatment is to commence
- special precautions where applicable
- date of prescription
- discharge and outpatient prescriptions require the quantity or duration of treatment
- each medication order must be signed by the prescriber
- each prescriber's name must be printed at least once on the WA HMC, and for every Schedule 4 Restricted and S8 prescription
 - where the order is made in EPMA software, the prescriber's signature is their HE number and password
- any alterations to a dose, frequency, rate or route must be signed and dated by the prescriber

2.1 Acceptable Terminology, Abbreviations and Symbols

Refer to the following guidelines for safe use of medicines terminology, acceptable terminology, abbreviations and symbols in prescribing:

- [Recommendations for safe use of medicines terminology November 2024](#)
- [Recommendations for safe use of medicines terminology Fact Sheet](#)
- [WATAG Acceptable Abbreviations](#)
- [List of Excluded Medicinal Items – active ingredient prescribing](#)
- [List of Medicines for Brand Consideration – active ingredient prescribing](#)
- **Write legibly** and in **BLOCK PRINT**, or via an approved Electronic Prescribing System.
- The active ingredient (generic name) for medicines should be used for all prescriptions. The only exception is insulin which should be prescribed by trade name only.
 - Trade names are preferred for insulin prescribing to avoid confusion, as there are many look-alike or sound-alike generic insulin names which are not interchangeable. Wherever possible, the patient should receive the brand of insulin that they use or will be using at home.
- The Australian Commission on Safety and Quality in Healthcare (ACSQHC) provides a [Active Ingredient Prescribing: List of Medicines](#), identifying medicines where brand name inclusion may be necessary for safe selection or clinical treatment. In cases where formulations are not interchangeable due to differences in active ingredient delivery, prescribers should annotate the brand name to ensure patient safety.
 - Prescribing by trade name alone is permitted in line with ACSQHC guidance on [Excluded Medicinal Items and Medicines for Brand Consideration](#).

- Avoid the use of chemical symbols (e.g. FeSO₄), lettered abbreviations (e.g. GTN) or chemical names (e.g. 6-mercaptopurine, which could result in six tablets being taken).

2.2 Patient Identification

All mandatory patient identification descriptors must be included on all handwritten and electronically generated prescriptions (paper, token or SMS). When using a patient addressograph on a medication chart or outpatient prescriptions the first prescriber must print the patient's name in the space provided below to document confirmation of the patient's identity (refer to Figure 1).

Figure 1: Patient Identification on the front and back of the WA HMC and Prescriber check for correct Addressograph

Affix patient identification label here and overleaf

URN: _____

Family name: _____

Given names: _____

Address: _____

Date of birth: _____ Sex: M ☐ F ☐

Medicare No: _____ PBS/RPBS Entitlement No. _____

☐ Concessional or dependent RPBS or Safety Net Concession Card Holder ☐ Safety Net Entitlement Card Holder

Print patient name here to confirm correct patient addressograph used

First prescriber to print patient name and check label correct:

Weight (kg): _____ Height (cm): _____ Date: ____/____/____

Figure 2: Patient Identification on handwritten hospital prescription and Prescriber check for correct Addressograph

UR number: _____ Ward: _____

Name: _____

Address: _____

DoB: _____

Fill in or attach the patient label

Print patient's name

Print patient name here to confirm correct patient addressograph used

Within electronic prescribing software, patient identifiers are automatically populated from webPAS and are visible on all screens, including all medication related screens used for prescribing, reviewing and administering medications.

2.3 Dosage Form and Strength

Where applicable, a prescription must specify the dosage form of the medication, for example:

- Specify formulation if applicable e.g. liquid/injection/ointment/cream/eye or ear drops.
- For inhalers, specify the type of inhaler device.
- For sustained release or extended-release medications, indicate modified release by ticking the slow-release box. (see Figure 3).

Figure 3: Slow Release Box on medication chart

Year 20..... DATE AND MONTH →

Doctors MUST ENTER administration times →

Start Date / / Medicine (print generic name)/form

Route Dose and Frequency and now enter times →

Indication Pharmacy Imprest S8 S4R

Prescriber signature Print name SAC/AAN

Tick if slow release

Tick Slow-Release box if formulation is a modified release product to indicate medication is to be swallowed whole and not crushed.

- The exact dose must be specified:
 - In the case of liquid medicines, the strength and the dose in milligrams or micrograms (not millilitres) must always be specified.
 - Specify the strength of the dosage form. Many medications are available in multiple strengths.

2.4 Dose, Frequency and Administration Times

- Write the route of administration clearly and use terms and abbreviations in accordance with [Recommendations for safe use of medicines terminology](#). For example, 'SL' is not an acceptable abbreviation for 'sublingual'.
- Write the weight (e.g. mg or microg), volume or units of a dose in full.
- Do not use symbols such as ½ - write 'half' or 'half a tablet'.
- For dose, avoid dangerous abbreviations such as 'mcg' or 'I.U', as they may be misinterpreted.
- Where possible avoid using decimals e.g. write 250 microg instead of 0.25 mg and trailing zeros e.g. 5 mg instead of 5.0 mg.
- Document the patient age and body weight on the WA HMC and prescription, as a safeguard for dosage calculations.
- When an unusually large dose of a medication is prescribed intentionally, that dose should be underlined and initialled by the prescriber.
- Best practice recommends that dose calculations involving intravenous medications be checked by a pharmacist, nurse or another doctor.
- Write in **English**. For frequency, avoid error prone abbreviations such as 'od', that could be misread as something else, instead use safe terms such a 'daily' or 'once a day'. (See Figure 4) Refer to ACHS [Recommendations for Safe Use of Medicines Terminology](#)
- On the WA HMC, ensure that the administration times are entered clearly and reflect the frequency stated.
 - Use the recommended administration times as stated within the WA HMC wherever clinically appropriate for a prescription.
- On a prescription, write adequate instructions in full.
 - Orders such as 'as directed' are not adequate for patients who forget, do not hear or do not understand verbal instructions. Prescriptions with 'as directed' will be returned for the prescriber to endorse with full directions.
- For prn medication, a frequency should be specified. When prescribing prn medication, consider how frequently it can be safely administered.
 - The frequency is a legal requirement for all Schedule 8 medicines.

Figure 4: Example of unacceptable abbreviations

'OD' is an unacceptable abbreviation as listed in the ACSQHC guidelines. In this example it has been misinterpreted as BD (twice daily) instead of daily

Start Date 10 / 11	Medicine (print generic name)/form Pantoprazole	Tick if slow release			
Route PO	Dose and Frequency 40mg OD	and now enter times →	08	X	AM
Indication Reflux	Pharmacy (impress) S8 S4R		20	PT	GO
Prescriber signature Doctor	Print name A. Doctor	SAC/AAN			

2.5 Route

- The route of administration must be clearly specified, and only approved abbreviations used as per the Australian Commission on Safety and Quality in Healthcare [Recommendations for safe use of medicines terminology](#).

Not all medicines are *bioequivalent* across routes of administration. Medicine orders, especially when doses are not bioequivalent should be for one route only. If multiple routes are likely to be needed the order for each route should be charted separately.

2.6 Instructions for administration

- Write instructions in full.
- Prescribe using only acceptable abbreviations (Refer to Section 2.1)

2.7 Quantity or length of treatment

- It is a legal requirement for Schedule 8 medicines that the **exact quantity** to be dispensed and repeat interval (if applicable), must be specified on the prescription.
- Over the counter medicines (OTC) prescribed via outpatient clinics may be rounded up to a full pack at the discretion of the dispensing outpatient pharmacist. This only applies to OTC medicines that haven't been prescribed as a finite course
- PBS discharge or outpatient prescriptions must meet PBS restrictions and include:
 - Specific quantities
 - Number of repeats where applicable (including Regulation 24 or 49)
 - Authority approval numbers
 - PBS prescriber number as outlined in the PBS Schedule. (See "PBS Prescription" for more information)
- Where appropriate, a specific length of treatment may be specified.

2.8 Telephone Order

A Medical Practitioner or Nurse Practitioner may verbally authorise the administration of a medicine within the scope of their professional practice. Verbal authorisation can only be provided for the administration of a single dose. Further administration must be following review of the patient. The final decision to accept a verbal order rests with the health professional administering the medication.

Telephone orders should only be issued in the following circumstances:

- Where possible, the order is given by a prescriber who is part of the team that normally cares for the patient, or the order is given by the duty doctor who has already reviewed the patient.
- The order is for continuation of therapy (except for intravenous antibiotics) and there has been no change in the patient's condition.

The details of the telephone order must be recorded on the front of the hospital medication chart and include:

- date and time prescribed
- name of the medicine
- dose to be administered
- route of administration
- prescriber's name
- initials of the two nurses to confirm that the verbal order has been independently received and read back to the prescriber
- time of administration

The order must be co-signed by the prescriber within 24 hours of the order AND ensure there are appropriate ongoing written orders for the patient as soon as practicable.

2.9 Changing a Medication Order on WA HMC

- Only medical practitioners, nurse practitioners and pharmacists are authorised to alter prescriptions.
- When stopping or changing a medication order, the original order on the WA HMC must not be obliterated.
- To cease or cancel a prescription:
 - A diagonal line across the prescription section, signature of the prescriber and reason the medication was ceased is required.
 - Taking care that the line does not impinge on other orders.
- Prescribers should initial / sign any changes on a prescription order.

Figure 5: Example of correct method to cease a medication order.

Figure 6: Example of **incorrect and unsafe changes to medication orders**

2.10 Day leave medication supply

Patients who are deemed suitable for leave (e.g. day leave) may have up to three days' supply of medicines prescribed. Medications for patients on leave are to be prescribed on the WA HMC in the appropriate area on the chart. (see figure 8)

Figure 8: Prescribing for Inpatients – leave medications

Continue on discharge? Yes / No		Continue on discharge? Yes / No		Continue on discharge? Yes / No		Continue on discharge? Yes / No	
Dispense? Yes / No		Dispense? Yes / No		Dispense? Yes / No		Dispense? Yes / No	
Duration:.....days Qty.....		Duration:.....days Qty.....		Duration:.....days Qty.....		Duration:.....days Qty.....	

your name: _____ Date: _____

2.11 Prescribing for Inpatients on Discharge

Discharge prescriptions should be written for medications which have been initiated during the admission, or wherever a change has been made (e.g. dose, frequency, form or combination product changes). Discharge prescriptions may be provided for additional medicines to:

- patients being discharged to a new residential care facility (see below).
- patients who are not returning immediately to their home on discharge who may not have access to their usual medication supply.
- or at the discretion of the clinical pharmacist.

A medication list should be reconciled with the WA HMC and the WA MMP to ensure completeness and accuracy of medicines information at discharge. All medication related information is to be included in the appropriate handover documents in line with WA Health Mandatory Policy [MP0095 Clinical Handover Policy](#).

Discharge prescriptions must be generated electronically via Notifications and Clinical Summaries (NaCS) and the medical discharge summary medication list must be completed at the time discharge prescriptions are generated.

2.11.1 The Discharge Summary

In all areas using Notification and Clinical Summaries (NaCS), the medication list must be completed when discharge prescriptions are generated. Refer to the [WA Health Department Clinical Handover Guideline](#)

2.11.2 Planned Discharges

Planned discharges (patients identified for discharge on the day prior or earlier) should have prescriptions written prior to 1600hrs at SCGH and 1400hrs at OPH on the day prior to discharge.

2.11.3 Unplanned Discharges

Patients identified for discharge on the same day should have all prescriptions written at the time they are identified for discharge and the clinical pharmacist notified, to allow for timely clinical review.

2.11.4 Discharges to Residential Care Facilities

Residential care facilities require a completed prescription and full medication list prior to discharge. If the patient is going to a new facility, prescriptions for ALL medications must be written. After review by the clinical pharmacist, prescriptions and medication lists are to be forwarded to the facility's nominated community pharmacy, and these posted directly to the community pharmacy.

2.11.5 Hospital provided Dose Administration Aids on discharge

Dose Administration Aids (ie Webster Paks®) may be supplied for patients who would otherwise be at high risk of medication misadventure and are unable to arrange one with their local community pharmacy. This service is only available during weekdays from 8:24 am to 3:30pm and at the discretion of the clinical pharmacist.

For more information, refer to:

[SCGOPHCG Patient Discharge and Discharge Planning](#)

[SCGOPHCG Dose Administration Aids](#)

3. Hospital Governance of Prescribing

3.1 WA Medicines Formulary

The [WA medicines formulary](#) is approved and maintained by The Western Australian Medicines Evaluation Panel (WAMEP) under [Statewide Medicines Formulary Policy MP 0077/18](#). Prescribers are required to adhere to the WA medicines formulary and its prescribing restrictions. Medicines on the WA medicines formulary are available according to the following categories.

See the [SCGOPHCG Medicines and Therapeutics Formulary Policy](#) and [SCGOPHCG Medicines Management Framework](#) for further details.

Formulary UNRESTRICTED

- Use is unrestricted, however only PBS quantities and repeats can be prescribed on discharge or for outpatients. When non-PBS subsidised medicines are prescribed in accordance with the SMF, a maximum of ONE month's supply with FIVE repeats (total of SIX months therapy) may be prescribed. Repeat supplies for these medications can be dispensed and supplied by the outpatient pharmacy.

Formulary RESTRICTED

- There are some restrictions in place, either a Formulary restriction or, PBS Restricted Benefit or PBS Streamlined Authority. PLEASE NOTE: If a patient requires treatment with a restricted medicine for an unlisted indication, an Individual Patient Approval (IPA) will be necessary. The clinical pharmacist or the local MTC should be consulted for guidance.

Formulary HIGHLY RESTRICTED

- These medicines require approval from a specialist prescriber type PRIOR to initiation.

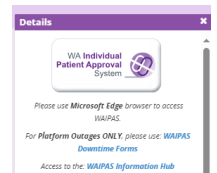
Not Approved by WAMEP

- Medicine has been reviewed at WAMEP and rejected for listing on the SMF. See the individual monograph on the SMF for further details.

NON-formulary

- These medicines are not available for the initiation of treatment. They have not been considered for listing on the formulary by the WA Medicines Evaluation Panel (WAMEP). Consult the local (MTC), as an IPA will be required to initiate therapy.
- When a medicine is assigned multiple traffic lights, it indicates that there are different approvals for various indications and/or formulations, each with distinct restriction levels.
 - See Figure 9 or the 'FAQs' link on the right-hand panel under Using [Formulary One](#) for more details.
- If a patient is admitted to the hospital while taking a non-formulary medicine, and the medicine has been deemed safe and necessary to continue, treatment should proceed for the duration of admission without consulting an MTC, to ensure continuity of care.
- Extemporaneous products are resource-intensive and often tailored to the specific needs of the patient. The availability of these products in the hospital will be determined by the MTC and the Pharmacy department.

Figure 9: Links via [Formulary One](#) to submit an application for Individual Patient Approval (IPA), and Statewide Medicines Formulary (SMF).



3.2 Special Access Scheme (SAS)

The Special Access Scheme allows health practitioners to access unapproved medicines for individual patients under specific conditions.

- At SCGOPHCG, a site-specific form is used for all Category A or Category C, SAS medication supplies: [SCGOPHCG Special Access Scheme \(SAS\) Medicine Form](#)
- The completed form must be returned to the Pharmacy department as per current processes before the medication can be supplied.
- The Pharmacy department will submit the form on behalf of the prescriber electronically on the [TGA Online Portal](#).
- For all Category B supplies, the supplier must apply and register on the [TGA SAS Portal](#).

See the [SCGOPHCG Medicines Management Framework](#) and [TGA SAS](#) for further details.

3.3 Antimicrobial Stewardship

- Antimicrobial agents are a commonly encountered restricted medication.
- Requests for approvals for use of restricted antimicrobials can be made by contacting the Infectious Diseases or Microbiology departments.
 - An approval code will be supplied and required for administration.
- Antimicrobial restriction policies are formulated by representatives of the Infectious Diseases Microbiology and Pharmacy departments which make up the Antimicrobial

Stewardship Team. They are responsible for the decisions regarding the optimal selection, dose and duration of an antimicrobial agent.

- The objective of the Antimicrobial Stewardship Team is to ensure the best clinical outcome for the treatment or prevention of infections, with minimal toxicity to the patient and minimal impact on subsequent resistance development.
- See the: [SCGOPHCG Antimicrobial Stewardship Policy](#)

3.4 Transfer of WA HMC and other medication charts between SCGH and OPH

Medication charts initiated at SCGH can be used at OPH and vice versa. There is no requirement to rechart medications upon transfer between the two sites.

Patients admitted from SCGH to OPH will have their current WA HMC, Anticoagulation Medication Chart and Insulin Subcutaneous Order and Blood Glucose Record Chart and Observation and Response Chart removed from their current SCGH file (at originator site) and placed in the OPH admission pack (admitting transfer site) by the ward clerk.

The admitting doctor is required to review the charts and record “OPH or SCGH reviewed”. In addition, they are required to clearly record the date, time of the review and sign legibly confirming the chart is appropriate for use at the new site.

Review to be documented:

- Medication Chart – front page, left hand side next to SCGOPHCG name
- Anticoagulation Chart – front page, top left hand margin
- Insulin Subcutaneous Order and Blood Glucose Record – inside chart (prescribing area) next to patient ID label

All medication orders must meet the requirements specified in this policy. Any medication orders that do not meet these requirements must be recharted prior to administration.

Nursing staff recording the first set of observations at OPH are required to notate above the date and time and sign legibly.

On discharge the ward clerk will photocopy the medication charts. Original charts will be returned to the receiving sites notes and the photocopies will be filed in the sending sites notes.

4.0 Monitored Medicines Prescribing Requirements

The [Monitored Medicines Prescribing Code](#) (MMPC) has replaced the Schedule 8 Medicines Prescribing Code. The MMPC details the requirements for prescribing Monitored Medicines in Western Australia and is published under the authority of the Medicines and Poisons Regulations 2016. The prescribing and dispensing of all Monitored Medicines in WA, must comply with the criteria and conditions listed in the MMPC.

Monitored Medicines include:

- All Schedule 8 (S8) medicines, also known as Controlled Drugs
- Schedule 4 (S4) Monitored Medicines (listed in Schedule 6 of the Regulation as S4 reportable medicines):
 - all benzodiazepines in S4,
 - codeine-based preparations in S4,
 - gabapentin,
 - pregabalin,
 - quetiapine,
 - tramadol,
 - zolpidem,
 - zolpiclone.

Key points regarding the MMPC:

- The MMPC strongly recommends the use of ScriptCheckWA prior to prescribing Monitored Medicines
 - A patient can be recorded as a 'drug dependent' or 'oversupplied' person on ScriptCheckWA
 - See Section 4.1 for further details
- For Monitored Medicines, the following information must be detailed on the prescription:
 - patient full name, address and date of birth,
 - date of prescription,
 - medicine, strength and form,
 - quantity for supply,
 - precise dosing directions,
 - number of times the prescription may be supplied
 - Name, address and contact telephone number of the prescriber,
 - Ink signature of the prescriber, for handwritten and computer-generated paper prescription
 - Additional requirements specific to S8 medicines:
 - A repeat interval if the S8 prescription is to be supplied more than once.
 - Each Schedule 8 medication must be written on a separate prescription to other prescribed medicines, unless the Schedule 8 medicines are the same medication in different formulations (e.g. Oxycodone IR and Oxycodone SR)
- S8 medication prescribing requirements:

Prescription Type	S8 Prescribing Requirements
WA HMC	Prescriber own handwriting
Handwritten outpatient prescriptions (used during downtime procedures only)	Prescriber own handwriting The patient's name, address and date of birth must be <i>handwritten</i> if addressograph is used
Computer-generated paper prescription, such as NaCS and eMedication	Electronically prescribed by a doctor, the prescription is printed onto a PBS paper prescription and <i>signed in ink</i> by a doctor

eRx tokens via eMedication Prescribing	Electronically generated and authorised with doctor He# and password
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- Prescribers may be contacted for verification of authenticity of a Monitored Medicine prescription and/or prescriber identity

4.1 ScriptCheckWA

ScriptCheckWA is Western Australia's Real Time Prescription Monitoring System and is a tool intended to assist in the safe prescribing of medicines, known to have a higher risk of dependence and diversion. It provides real time information that can help inform risk assessment, including prescriptions issued, prescriptions dispensed, authorisations issued to prescribers, and alerts relating to higher-risk criteria. This information can assist in assessing safety of prescribing Monitored Medicine(s) for a patient. All prescribers and pharmacists are required to [Register for ScriptCheckWA](#).

In the discharge and outpatient setting prescribers are strongly recommended to check ScriptCheckWA prior to prescribing Monitored Medicines. ScriptCheckWA links are built into a number of hospital health applications including; ePrescribing (ClinicalWorkBench) and iClinical Manager.

Prescriptions issued via eMedication (i.e. ClinicalWorkBench) are automatically recorded in ScriptCheckWA. Handwritten prescriptions would require manual entry into ScriptCheckWA.

4.2 MMPC in the hospital setting

The MMPC allows certain exemptions to the prescribing of Monitored Medicines within the hospital setting. Table 4.1 represents a summary of exemptions and authorisations which apply to different schedule and medication classes within the MMPC. Refer to the [Monitored Medicines Prescribing Code](#) for detailed and specific information on the conditions which apply to the prescribing of each class of Monitored Medicines.

4.2.1 Inpatient prescribing

The requirements of the MMPC do not apply to the prescribing of Monitored Medicines to inpatients in hospitals for continuing treatment or patients who require treatment with monitored medicine(s) in relation to their episode of care, excluding MDMA, psilocybine, esketamine.

4.2.2 Discharge prescribing

For Monitored Medicines excluding opioid pharmacotherapy, stimulant medicines, esketamine, MDMA and psilocybine a **prescription of a 14-day supply of the monitored medicine(s) is permissible without additional authorisation or documentation from emergency departments or on discharge from hospital where;**

- I. the treatment with the monitored medicine is associated with that episode of care and
- II. the patient is not recorded as "Drug Dependent" or "Oversupplied" in ScriptCheck WA

4.2.3 Outpatient prescribing

The MMPC defines the circumstances in which a prescriber for a specific patient:

- i. Can prescribe a Monitored Medicine without prior authorisation from the WA Health Chief Executive (i.e. the Director General)
- ii. Requires prior authorisation from the WA Health Chief Executive before prescribing a Monitored Medicine



MMPC Summary Table 4.1

The below table summarises the hospital exemptions within the MMPC and the actions to be undertaken when prescribing monitored medicines in each setting, including where prescribing may only occur by approved specialist, where additional authorisations are required and where exemptions to outpatient prescribing exist.

Monitored Medicine Class or Patient Population	Inpatient prescribing	Discharge prescribing (up to 14 days supply)	Discharge prescribing (greater than 14 days supply)	Outpatient prescribing	Approved Specialist to Prescribe	Authorisation(s) required	Exemptions to Outpatient Prescribing
Schedule 4 Monitored Medicines	✓	✓	Check SCWA Refer to MMPC Part 8 Document a plan to mitigate potential harms in specific circumstances, refer to Part 8.5 of MMPC		NO	NO	N/A
S8 Opioids (<u>NOT</u> including Ketamine and Methadone)	✓	✓*	Check SCWA Refer to MMPC Part 2		Specific circumstances, refer to Sections 2.5, 2.6 of the MMPC		End of Life care exemption 14-day specialist exemption
Ketamine	✓	✓*	Check SCWA Refer to MMPC Part 2		YES Pain or Palliative	Refer to MMPC Part 2 and Figure 2	End of Life care exemption 14-day specialist exemption

*If patient is not recorded as Drug Dependent or Oversupplied in SCWA and Rx is related to that episode of care

Methadone (non-opioid pharmacotherapy indication)	✓	✓*	Check SCWA Refer to MMPC Part 2		YES (see Table 2 in MMPC)	Refer to MMPC Part 2 and Figure 2	14-day specialist exemption
Opioid Pharmacotherapy	✓	✗	✗	✗	YES CPOP Prescriber	YES	None
Stimulant Medicines	✓	✗	✗	Check SCWA Refer to MMPC Part 3	YES	Refer to MMPC Part 3	None
Medicinal Cannabis	✓	✓*	Check SCWA Refer to MMPC Part 4		Refer to MMPC Part 4		
S8 benzodiazepines, sodium oxybate and all other S8's not captured elsewhere in the MMPC	✓	✓*	Written authorisation from the CEO is always required		YES Refer to MMPC Part 6	Written authorisation from CEO is required for all outpatient and discharge supply>14 days	None
MDMA & Psilocybine	Written authorisation from the CEO is always required Refer to MMPC Part 5						
Esketamine	Restricted to psychiatrists treating patients in particular clinics				YES (Psychiatry)	Refer to MMPC Part 5	
Drug Dependent or Over Supplied Person	✓	CEO Approval Required for S8 Medicines	Refer to MMPC		Refer to MMPC		

***If patient is not recorded as Drug Dependent or Oversupplied in SCWA and Rx is related to that episode of care**



4.3 Authorisation from the CEO to prescribe a S8 medicine

In certain circumstances written authorisation from the CEO is required before prescribing a S8 Medicine. The specific circumstances where this is required and related processes are detailed in the MMPC. The below list represents common circumstances when Outpatient prescribing of Monitored Medicine(s) at SCGOPHCG requires authorisation from the CEO.

- Prescribing S8 benzodiazepines
- Prescribing Sodium Oxybate
- Prescribing for a person recorded as drug dependent, oversupplied and/or with a history of substance abuse (within the previous 5 years)

Application forms to the DOH CEO can be found at "[Monitored Medicines](#)". The CEO will notify applicants in writing or electronically of approval or rejection of an application. Where ScriptCheckWA displays an authorisation to prescribe a specified S8 medicine(s), only the practitioner(s) named or, where relevant, practitioners practicing at the clinic named on the authorisation are permitted to prescribe these medicine(s).

4.4 Approved Specialists

Certain S8 Medicines can only be prescribed by approved specialists under certain circumstances, which includes:

- S8 stimulant medicines
- Methadone, for non-opioid pharmacotherapy indication
- Ketamine (pain or palliative care specialist)
- Esketamine (Psychiatrists)

For other Monitored Medicines which may require prescribing by approved specialists, consult MMPC.

5.0 Pharmaceutical Benefit Scheme

5.1 PBS Prescriptions

SCGOPHCG is authorised to issue PBS prescriptions and is required to adhere to PBS prescription requirements.

- Outpatients can be prescribed PBS prescriptions for a quantity up to the PBS maximum.
- Inpatients may be prescribed up to one-month supply on discharge.
 - Repeats should not be written unless there is a specific course of treatment that the patient needs at discharge (e.g. antibiotics).
- Up to 10 medications may be prescribed on one prescription *except*:
 - For Schedule 8 medicines that require a separate prescription.
- Both PBS and non-PBS items may be written on the same prescription.
- Community pharmacists may dispense and claim for public hospital PBS prescriptions (excluding HSD).

5.2 Tips for writing PBS prescriptions:

- Check if the item requires a PBS authority. (See “Authority Items”)
- Schedule 8 medications must have the exact quantities and repeats written on the prescription. (See “Schedule 8 Medication Prescribing Requirements”)
- A prescriber’s name and prescriber number must be on the prescription.
 - Prescribers must be registered with Medicare and have a Medicare prescriber number. (This is **not** the same as the provider number)
 - To confirm your prescriber number call Medicare Australia on 132 150.
- A prescriber cannot write more than one PBS benefit (medication) for the same medication, for the same person on the same day.
- Any alterations on the prescription need to be signed by the prescriber.
- A Pharmacist is authorised to annotate any clarifications to an order, including a decrease in quantity where this is appropriate,
- If using an addressograph sticker on a hospital PBS prescription, the prescriber must also handwrite the patient’s name.
- For a S8 prescription (discharge or outpatient) the patient’s name, address and date of birth must be handwritten unless the prescription is generated by NaCS or eMedication.

5.3 PBS Authority Items:

- Authority items are those that the PBS will only cover if specific criteria are met.
 - Authorities may also be required for quantities greater than those set in the PBS schedule.
- The prescription must have an authority number, the phone approval code or streamlined authority code written on the prescription before it can be dispensed.
- There are TWO types of Authorities:

1. **Authority STREAMLINED**

- Do not require a telephone or written approval from Medicare.
- Prescribers record the five-digit PBS code for the relevant indication directly on the prescription.
- The streamlined code must match the prescribing criteria and cannot exceed the listed quantity or repeats for the Streamlined Authority.

2. **Authority REQUIRED**

- Requires approval from Medicare prior to use. This may occur via written, telephone or online platforms i.e. PRODA.
- If a patient requires a larger quantity and repeats on a PBS medication, the prescriber can apply for an Authority for increased quantity and repeats by contacting Medicare.
- Authority numbers can be obtained by calling 1800 888 333 (*4PBS) for general patients and 1800 552 580 for Repatriation patients (Veterans Affairs).
- When calling for Authority numbers, make sure you have the following ready;
 1. Prescription number (top left of script)
 2. Patient’s name and address
 3. Patient’s Medicare number (can be obtained on i.Clinical Manager)
 4. The PBS criteria that the patient meets
 5. Your prescriber number

Other information:

- If an item is restricted, but the patient does not meet the specified criteria, it cannot be prescribed as a pharmaceutical benefit. The prescriber should endorse the prescription with 'non-PBS' to denote to the dispensing pharmacists it cannot be claimed as a PBS benefit.
- Veterans can access a broader range of medications under the RPBS schedule.
- Patients will be provided with the brands of medications stocked at SCGOPHCG, alternatively they can obtain medication from a community pharmacy.
- Items with specific administration routes cannot be used differently as pharmaceutical benefits (e.g., ophthalmic preparations cannot be prescribed for topical skin use).

5.4 Electronic Prescribing on eMedication

eMedication is a platform for Electronic Prescribing and Medication Administration (EPMA) that some clinical areas at SCGOPHCG are utilising. Resources on eMedication:

- [eMedication eLearning](#)
- [Electronic Prescriptions Overview](#)
- [Electronic Prescribing FAQs](#)
- [eMedication User Guide](#)
- [eAllergy User Guide](#)

5.5 Downtime procedure for electronic prescribing:

In the event of electricity failure or information technology failure required for eMedication, prescribers are to prescribe on PBS prescription or NaCS generated prescription.

6.0 Non-Medical Prescribers

6.1.1 Nurse Practitioners

Nurse Practitioners are credentialled by the NMHS Nurse Practitioner/Endorsed Privately Practising Midwives Committee.

6.1.2 Endorsed Optometrists

Optometrists with a scheduled medicines endorsement are qualified to prescribe an Optometry Board-approved list of topical schedule 2,3 and 4 medicines for use in the practice of optometry. For a list of scheduled medicines endorsed Optometrists can prescribe, see the Optometry Board of Australia; Guidelines for use of scheduled medicines Appendix B.

6.1.3 Endorsed Podiatrists

Eligible podiatrists with scheduled medicines endorsement at SCGOPHCG can prescribe and administer medicines as per Appendix 1: Medicines Prescribing List - Eligible Podiatrists with Scheduled Medicines Endorsement

6.1.4 Dieticians

Dieticians at SCGOPHCG can prescribe nutritional supplements as per the SCGOPHCG Clinical Practice Guideline “Charting of Nutritional Supplements”, which authorises prescribing of nutritional supplements on the WA HMC.

6.1.5 Pharmacist charting

All pharmacist charted medications must be documented in the appropriate section of the WA hospital medication chart (WA HMC) (MR 810.10).

For more information, refer to:

- [SCGOPHCG Partnered Charting Pharmacist Procedure](#)
- [SCGOPHCG Pharmacist Initiated Medicines \(PIMS\) Procedure](#)

6.1.6 Structured Administration and Supply Arrangements (SASA)

Structured Administration and Supply Arrangements (SASA) allow a health professional without prescribing rights to initiate administration or supply of a medicine in the circumstances described in the SASA. Hospital or health services may issue a SASA with endorsement from the MTC and appropriate delegated authority.

Refer to: [SCGOPHCG Structured Administration and Supply Arrangements \(SASAs\) Policy](#).

6.1.7 Advanced Scope Physiotherapists

Advanced Scope Physiotherapists authorised by the SCGOPHCG Medicines & Therapeutics Committee, can administer certain medicines defined in the professions SCGOPHCG SASA's and must document administration on the WA HMC “once only” section.

6.1.8 Nurse-Initiated Medicines

A registered nurse or medicines-competent enrolled nurse may give approved Schedule 2 and Schedule 3 non-prescription and non-scheduled medications as per the [SCGOPHCG SASA – Nurse Initiated SASA](#)

The nurse will document this medicine under the ‘once only, pre-medication and nurse/midwife-initiated medicines’ section on the front of the WA HMC and comply with all legal requirements. The nurse will document the rationale for initiating the medication in the inpatient progress notes, and if the medication is to continue after 24 hours it must be prescribed by a medical officer.

Related Documents

- [State Medicines Formulary](#)
- [Pharmaceutical Benefits Scheme \(PBS\)](#)
- Australian Commission on Safety and Quality in Health Care. Action 4.10: Medication Review. Available from: <https://www.safetyandquality.gov.au/standards/national-safety-and-quality-health-service-nsqhs-standards/medication-safety-standard/continuity-medication-management/action-410>

Western Australian Legislation:

- [Medicines and Poisons Act 2014](#)
- [Medicines and Poisons Regulations 2016](#)

Western Australian Department of Health Mandatory Policies:

- [High Risk Medication Policy](#)
- [Medication Chart Policy](#)
- [Medication Review Policy](#)
- [Statewide Medicines Formulary Policy](#)

References

1. Women and Newborn Health Services. [WNHS Prescribing Procedure](#)
2. Fiona Stanley Fremantle Hospitals Group. [FSH Prescribing policy and procedure](#)
3. Sir Charles Gairdner Osborne Park Hospital Care Group. [SCGOPHCG Medication Management Policy and Procedure.](#)
4. Sir Charles Gairdner Osborne Park Hospital Care Group. [SCGOPHCG Management of Prescription Stationery Policy](#)

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.



Appendix 1. Medicines Prescribing List - Eligible Podiatrists with Scheduled Medicines Endorsement

The following medicines as approved by the Podiatry Board of Australia may be prescribed and administered by eligible podiatrists with scheduled medicines endorsement at SCGOPHCG.

Medication	Route	Indication	Maximum duration of supply
Anaesthetics and analgesics			
Bupivacaine plain or with adrenaline (epinephrine)	Subcutaneous injection	Local anaesthesia for ingrown toenail surgery, flexor tenotomy or tissue biopsy	Single dose
Lidocaine (lignocaine) plain or with adrenaline (epinephrine)	Subcutaneous injection	Local anaesthesia, for ingrown toenail surgery, flexor tenotomy or tissue biopsy	Single dose
Prilocaine	Subcutaneous injection	Local anaesthesia, for ingrown toenail surgery, flexor tenotomy or tissue biopsy	Single dose
Ropivacaine	Subcutaneous injection	Local anaesthesia, for ingrown toenail surgery, flexor tenotomy or tissue biopsy	Single dose
Aspirin	Oral	Analgesia	One pack supply
Paracetamol	Oral only	Analgesia	One pack supply
Antimicrobials			
*Supply note: Antimicrobials should be prescribed in accordance with recommendations in the Therapeutic Guidelines and supply be limited to one pack with a short review period. Repeats may be considered for patients requiring follow-up in the Multidisciplinary Foot Ulcer Clinic with total supply reflecting sufficient quantities to cover the patient until their appointment.			

Amoxicillin with clavulanic acid	Oral only	Soft tissue infection associated with diabetic foot ulcer	See supply note*
Cefalexin	Oral	Soft tissue infection associated with diabetic foot ulcer	See supply note*
Ciprofloxacin	Oral only	Soft tissue infection associated with diabetic foot ulcer	See supply note*
Clindamycin	Oral only	Soft tissue infection associated with diabetic foot ulcer	See supply note*
Flucloxacillin	Oral only	Soft tissue infection associated with diabetic foot ulcer	See supply note*
Metronidazole	Oral only	Soft tissue infection associated with diabetic foot ulcer	See supply note*
Antifungals			
Clotrimazole <i>Note: does not include hydrocortisone with clotrimazole combination product.</i>	Topical	Tinea pedis, cutaneous candidiasis	One pack supply
Econazole	Topical	Tinea pedis, cutaneous candidiasis	One pack supply
Nystatin	Topical	Tinea pedis, cutaneous candidiasis	One pack supply
Corticosteroids			
Betamethasone	Topical or injection	Podiatric inflammatory conditions Injection limited to treatment of podiatric conditions where there is evidence or best practice recommendations to support its use	One pack supply or Single dose (injection)
Dexamethasone	Injection only	Podiatric inflammatory conditions Injection limited to treatment of podiatric conditions where there is evidence or best practice recommendations to support its use	Single dose

Hydrocortisone <i>Note: does not include hydrocortisone with clotrimazole combination product.</i>	Topical	Podiatric inflammatory conditions	One pack supply
Methylprednisolone	Topical or Injection	Podiatric inflammatory conditions Injection limited to treatment of podiatric conditions where there is evidence or best practice recommendations to support its use	One pack supply or Single dose (injection)
Mometasone	Topical	Podiatric inflammatory conditions	One pack supply
Triamcinolone	Topical or injection	Podiatric inflammatory conditions Injection limited to treatment of podiatric conditions where there is evidence or best practice recommendations to support its use	One pack supply or Single dose (injection)
Non-steroidal anti-inflammatories			
Diclofenac	Oral	Analgesia	One pack supply
Ibuprofen	Oral	Analgesia	One pack supply
Indometacin	Oral	Analgesia	One pack supply
Naproxen	Oral	Analgesia	One pack supply
Sympathomimetics			
Adrenaline (epinephrine)	Intramuscular Injection only	Anaphylaxis	Single dose